

Policy on Imaging Services

«Hospital Name»

Document control

Document No.	HCO/AAC/POL/08	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Imaging In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

Imaging services are provided as per the scope of services of the organisation — complying with legal requirements, available when clinical care needs them, reported on time, and quality-assured when outsourced.

1. Purpose

This policy says how «Hospital Name» provides imaging services that comply with legal requirements (including Atomic Energy Regulatory Board (AERB) clearance and Pre-Conception and Pre-Natal Diagnostic Techniques (PC-PNDT) duties), keeps scope, infrastructure and human resources commensurate with clinical services, uses qualified personnel to perform, supervise and interpret investigations, delivers results within defined turnaround time (TAT) while monitoring waiting, performance and report times, intimates critical results immediately, with read-back, reports in a standardised manner (including teleradiology naming), recalls or amends reports when needed, and outsources unavailable tests under a quality-assured MoU.

The chapter intent is that imaging services are reliable, timely, legally compliant and continuous with patient care.

This policy does not cover imaging quality assurance, radiation safety programmes, patient screening before imaging, or informed consent for imaging procedures — those are covered in other hospital policies.

2. Scope

This policy applies to the imaging/radiology in-charge, Radiation Safety Officer (RSO), radiographers and technologists, reporting doctors (including teleradiology reporters), treating doctors who request imaging, nurses who accompany patients, and the Quality Coordinator at «Hospital Name».

It covers imaging service provision: legal and regulatory compliance (including Atomic Energy Regulatory Board (AERB) and PC-PNDT obligations); clinical scope; infrastructure and human resources with round-the-clock availability or safe-transfer arrangements when outsourced; qualified performance, supervision and interpretation; turnaround time monitoring; critical-result intimation; standardised reporting; recall and amendment; and outsourcing arrangements with MoU.

Boundaries:

- The imaging QA programme, radiation-safety programme, patient screening before imaging, radiation-safety devices, training and signage are covered in other hospital policies. This policy owns legal licences, service scope, TAT, critical intimation, reporting format and outsourcing MoUs.
- Appropriateness of accompanying staff during transfer is covered in other hospital policies; safe transfer of patients to outsourced imaging is required here as part of availability.
- MoU quality content is covered in other hospital policies; this policy requires an MoU incorporating quality assurance for imaging outsourcing.

3. Policy standards

«Hospital Name» provides imaging services that comply with legal and other requirements, including AERB clearance, dosimeters, lead shields and aprons, PC-PNDT displays and reports, and a Radiation Safety Officer of appropriate level. Scope, infrastructure and human resources are commensurate with clinical services; imaging is available round the clock, with safe transfer and timely reports when a modality is outsourced. Qualified and trained personnel perform, supervise and interpret investigations. Results are available within defined TAT with monitoring of waiting, performance and report times. Critical results are intimated immediately on availability; read-back is confirmed and documented. Reports follow a standardised format including teleradiology naming. There is a mechanism to recall or amend reports. Tests not available in-house are outsourced only under a quality-assured MoU.

«Hospital Name» does not operate radiation imaging without current legal compliance, and does not delay communication of a critical imaging result once the report is ready.

4. Non-negotiable rules

1. Do not operate radiation-emitting imaging equipment without current AERB clearance and a named Radiation Safety Officer of appropriate level.
2. Do not operate ultrasound used for prenatal diagnosis without current PC-PNDT registration, required displays and reports to the competent authority.
3. Do not report an imaging examination without qualified interpretation as required for the modality.
4. Do not leave a critical imaging result uncommunicated once the report is ready.
5. Do not alter or modify content of an outsourced imaging report.
6. Do not outsource imaging to a provider that has no documented quality assurance system and no MoU with this hospital.
7. Do not leave a recalled report in clinical areas, the medical record, RIS or HIS after recall without replacement by the amended report.
8. Do not proceed with imaging when required legal compliance checks for the modality are missing — stop-work applies.
9. Staff who see an imaging service rule broken report it the same shift to the Imaging In-Charge, the Radiation Safety Officer or the Medical Superintendent.

5. What we do

5.1 Legal and other requirements

Imaging services comply with legal and other requirements. «Hospital Name» is aware of legal and other requirements for imaging, documents them for information and compliance, and maintains and updates compliance status regularly.

Statutory requirements met include: AERB clearance; dosimeters; lead shields; lead aprons; signage; display as per the Pre-Conception and Pre-Natal Diagnostic Techniques (Prohibition of Sex Selection) Act, 1994; reports to the competent authority; and a Radiation Safety Officer (RSO) of appropriate level.

The imaging in-charge and RSO maintain a register of licences, registrations, RSO appointment and renewal dates. Renewal is initiated well in advance of expiry. Equipment is not used when clearance has lapsed — stop-work applies.

5.2 Scope of imaging services

Scope of the imaging services is commensurate with the services provided by «Hospital Name». Example from the standard: an organisation providing neurosurgery services including head injuries shall have facilities for CT scan.

Services are available round the clock so that patient care is not disrupted. Imaging modalities required for emergency management are preferably available within the premises. The imaging in-charge maintains a service menu of in-house and outsourced

modalities, reviewed at least annually (the organisation's adopted cycle) and whenever clinical services change.

5.3 Infrastructure and human resources

The infrastructure (physical and equipment) and human resources are adequate to provide for the defined scope of services. Imaging has adequate space and equipment; reports must not be delayed for lack of equipment or human resources, including personnel authorised to report results.

Round-the-clock availability is organised by roster for in-house modalities and by documented safe-transfer and turnaround arrangements for outsourced emergency modalities (see section 5.9). Staffing and authorised reporters are reviewed at least annually (the organisation's adopted cycle) against workload.

5.4 Qualified performance, supervision and interpretation

Qualified and trained personnel perform, supervise and interpret the investigations. AERB guidelines are used as a reference document for radiation-based imaging.

The imaging in-charge keeps competency records for radiographers/technologists and the list of doctors authorised to interpret and report each modality, including teleradiology reporters. Unauthorised staff do not release reports.

5.5 Results within defined turnaround time

Imaging results are available within a defined time frame. The organisation documents TAT of imaging results for all modalities and monitors waiting times, time taken to perform the tests, and time taken to prepare the reports for all modalities.

The defined timeframes may differ for different tests and are decided based on the nature of the test, modality, criticality and urgency required by the treating doctor. The imaging in-charge documents the defined TAT for each modality in the imaging service menu or a separate TAT schedule.

The imaging in-charge reviews waiting, performance and report-time data at least monthly — the organisation's adopted monitoring cycle; NABH requires performance monitoring without specifying a minimum frequency.

5.6 Critical imaging results

Critical results are intimated immediately to the personnel concerned. The organisation defines and documents critical results that require immediate attention for patient management for each modality. Critical results of outsourced investigations are also intimated.

Critical results are communicated to a person from the treating team as soon as the test result or report is ready. The intimation includes: name of the patient; unique ID; date and

time of imaging; investigation name and result; identity of who communicated; identity of the recipient; read-back; and date and time of acknowledgement.

5.7 Standardised reporting

Results are reported in a standardised manner. At a minimum the report includes: the name of the organisation (or outsourced imaging centre); the patient's name; the unique identification number; and the name and signature of the person reporting the result.

For teleradiology, the report includes the name of the reporting doctor and a remark to that effect, and the name of the reporting organisation if outsourced to an organisation. The hospital does not alter or modify anything in the outsourced report.

5.8 Recall and amendment of reports

There is a mechanism to address the recall or amendment of reports whenever applicable. When a report is recalled, withdrawal from clinical areas, medical records, RIS and HIS is ensured. If already issued to the patient, the amended report is made available with a caution to ignore the earlier one.

5.9 Outsourced imaging tests

Imaging tests not available in the organisation are outsourced to organisation(s) based on their quality assurance system. Written guidance includes: list of tests for outsourcing; identity of personnel in outsourced facilities to ensure safe transportation of patients and completion of imaging results; manner of identification of patients and test requisition details; methodology to check selection and performance of the outsourced facility; reporting of critical results; TAT for emergency and routine requests; and prioritisation for urgent investigations.

The organisation has an MoU/agreement that incorporates quality assurance and the requirements of this standard. The panel and MoUs are reviewed at least annually (the organisation's adopted cycle).

6. Stop-work authority

Do not scan or expose a patient when required legal compliance for the modality is missing — AERB clearance and RSO cover for radiation equipment; PC-PNDT registration and displays for covered ultrasound.

Do not use radiation-emitting equipment when mandatory personnel monitoring (dosimeter/TLD) or required lead protection for the examination is unavailable.

Stop-work applies to the imaging examination, not to emergency clinical stabilisation outside the imaging room.

The person who stops tells the Imaging In-Charge or the Radiation Safety Officer the same shift, and the Medical Superintendent if legal clearance is the barrier. Refusing to scan without required checks is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that imaging services are legally compliant, available as scoped, and meet TAT and critical-result duties.
Imaging / Radiology In-Charge	Maintains service menu, licence register (with RSO), staffing, TAT definitions, critical-result list, report format, recall mechanism and outsourced panel with MoUs.
Radiation Safety Officer (RSO)	Holds AERB-facing compliance for radiation equipment, dosimetry and related statutory duties under this policy's legal section; coordinates with the hospital's radiation-safety programme.
Radiographers / technologists	Perform examinations they are trained for; monitor times; communicate critical results when designated; do not alter outsourced reports.
Reporting doctors (including teleradiology)	Interpret and report within authorisation; support recall/amendment.
Treating doctors and accompanying nurses	Request appropriately; complete transfer safely when imaging is off-site; receive critical-result intimation with read-back.
Quality Coordinator	Audits this policy; tracks CAPA when TAT, critical-result or legal-compliance defects recur.

8. Quality monitoring (RCA → CAPA)

What is monitored:

- AERB clearance, PC-PNDT registration, RSO appointment and dosimeter/lead-protection status current.
- Service menu commensurate with clinical services; round-the-clock arrangements documented.
- Infrastructure and staffing adequate; authorised reporters listed.
- Waiting, performance and report-time monitoring against defined TAT.
- Critical-result communication immediately, with read-back documentation.
- Standardised reports including teleradiology naming; no alteration of outsourced content.
- Recall/amendment log complete with RIS/HIS withdrawal evidence.

- Outsourced MoUs current and quality-based.

Root-cause analysis is required when the same imaging service defect recurs; the organisation's defined recurrence threshold applies, as documented in the hospital's Root Cause Analysis procedure.

This policy is reviewed annually, and sooner when modalities, legal registrations or outsourcing arrangements change.

9. Training and staff acknowledgement

All imaging staff, RSO, nurses who accompany patients to imaging and doctors who request or report imaging are trained on this policy at induction and once a year after that. Training covers legal compliance triggers for stop-work, TAT monitoring, critical-result intimation with read-back, standardised reporting, recall/amendment and outsourcing rules.

Staff acknowledgement

I have read the Policy on Imaging Services of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

10. Distribution

Official master copy: office of the Medical Superintendent, «Hospital Name», with the Quality Coordinator.

Copies issued to: imaging/radiology; RSO; emergency; every in-patient ward; out-patient; nursing administration; intensive-care areas if present.

The current version is available to all staff at the front-office policy file and, if the hospital keeps an intranet, at staff intranet / policies.

When a new version is issued, take old copies out of use.

11. Abbreviations

Abbreviation	Meaning
AAC	Access, Assessment and Continuity of Care

	(NABH HCO chapter)
AERB	Atomic Energy Regulatory Board
CAPA	corrective and preventive action
CT	computed tomography
HCO	Hospital (Full Accreditation programme)
HIS	Hospital Information System
MoU	Memorandum of Understanding
MRI	magnetic resonance imaging
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	objective element
PC-PNDT	Pre-Conception and Pre-Natal Diagnostic Techniques (Prohibition of Sex Selection) Act, 1994
PRE	Patient Rights and Education
RIS	Radiology Information System
ROM	Responsibilities of Management
RSO	Radiation Safety Officer
TAT	turnaround time
TLD	thermo-luminescent dosimeter

12. Traceability to NABH HCO Full Accreditation 6th Edition AAC.8

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
AAC.8.a	CORE	Imaging services comply with legal and other requirements.	Section 3; 5.1; Section 6 Stop-work	Imaging In-Charge and RSO (maintain compliance); Medical Superintendent (accountable)
AAC.8.b	Commitment	Scope of the imaging services is commensurate to the services provided by the organisation.	Section 3; 5.2	Imaging In-Charge (maintain menu); Medical Superintendent (approve)
AAC.8.c	Commitment	The infrastructure (physical and equipment) and human resources are	Section 3; 5.3	Imaging In-Charge (manage); Medical Superintendent (resource)

		adequate to provide for its defined scope of services.		
AAC.8.d	Commitment	Qualified and trained personnel perform, supervise and interpret the investigations.	Section 3; 5.4	Imaging In-Charge (competency records); reporting doctors (interpret); radiographers (perform)
AAC.8.e	Commitment*	Imaging results are available within a defined time frame.	Section 3; 5.5	Imaging In-Charge (define and monitor); Quality Coordinator (audit)
AAC.8.f	Commitment*	Critical results are intimated immediately to the personnel concerned.	Section 3; 5.6	Designated imaging personnel (intimate); treating team (read-back); Imaging In-Charge (critical list)
AAC.8.g	Commitment	Results are reported in a standardised manner.	Section 3; 5.7	Imaging In-Charge (format); reporting doctor (sign); staff (no alteration of outsourced content)
AAC.8.h	Commitment*	There is a mechanism to address the recall / amendment of reports whenever applicable.	Section 3; 5.8	Imaging In-Charge (run mechanism); Quality Coordinator (CAPA)
AAC.8.i	Commitment	Imaging tests not available in the organisation	Section 3; 5.9	Imaging In-Charge (panel and MoU); Medical

		are outsourced to organisation(s) based on their quality assurance system.		Superintendent (approve MoU)
--	--	--	--	------------------------------

13. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

AAC.8.a — Legal and other requirements

- Register of AERB clearances, dosimeters, lead protection and RSO appointment.
- PC-PNDT registration, displays and reports to competent authority where applicable.
- Periodic update of legal-compliance status.

AAC.8.b — Scope commensurate with services

- Imaging service menu listing in-house and outsourced modalities.
- Annual review of menu against clinical services.
- Round-the-clock availability / safe-transfer arrangement note.

AAC.8.c — Infrastructure and human resources adequate

- Equipment inventory linked to the maintenance/QA programme.
- Staff and authorised-reporter list with shift cover.
- Annual adequacy review of space, equipment and human resources.

AAC.8.d — Qualified performance, supervision and interpretation

- Qualification and training records for imaging personnel.
- List of doctors authorised to interpret/report by modality (including teleradiology).
- Note of AERB guidance used as reference for radiation-based imaging.

AAC.8.e — Results within defined TAT

- Defined TAT for all modalities.
- Monitoring records of waiting, performance and report times for IP/OP/emergency.
- Monthly TAT exception/escalation records.

AAC.8.f — Critical results intimated immediately

- Documented critical-result definitions per modality.
- Critical-result communication log with patient ID, result, caller, recipient, read-back, date and time.
- List of personnel authorised to report critical imaging results.

AAC.8.g — Standardised reporting

- Standardised report template with minimum required fields.
- Teleradiology reports naming reporting doctor and organisation as required.
- Audit finding that outsourced report content was not altered.

AAC.8.h — Recall / amendment mechanism

- Recall/amendment log with reason and date.
- Evidence of withdrawal from clinical areas, medical records, RIS and HIS.
- Amended report issued to patient with caution where previously issued; CAPA record.

AAC.8.i — Outsourced imaging tests

- Written outsourcing guidance including critical-result and TAT/prioritisation rules.
- Current MoU/agreement incorporating quality assurance.
- Annual review of outsourced imaging performance.

14. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Guidebook to Accreditation Standards for Hospitals, 6th Edition — Access, Assessment and Continuity of Care (AAC), standard AAC.8.
- Atomic Energy (Radiation Protection) Rules, 2004 — licensing, radiation protection and related duties for radiation-emitting imaging.
- Pre-Conception and Pre-Natal Diagnostic Techniques (Prohibition of Sex Selection) Act, 1994 — registration, display and reporting for covered prenatal diagnostic imaging.
- AERB guidelines — reference for radiation-based imaging practice (AAC.8.d).

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

Several requirements in this document are statutory rather than advisory — in particular those arising under the Atomic Energy (Radiation Protection) Rules, 2004, and the Pre-Conception and Pre-Natal Diagnostic Techniques (Prohibition of Sex Selection) Act, 1994. Statutory requirements change, and State authorities impose additional or stricter conditions. «Hospital Name» is responsible for verifying the current text of any rule cited here and the conditions attached to its own authorisations and licences; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

This document is not issued by, endorsed by, or affiliated with NABH, the World Health Organization, the National Centre for Disease Control, the Food Safety and Standards Authority of India, any Pollution Control Board, or any other body named in it. Wording is original; no text has been reproduced from the standards, rules or guidelines referenced.